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9

Psychiatric Responsibility and the Violent Patient

Overview of the Law

As a general rule, in the absence of a special relationship (e.g., jailer–prisoner), one person has no legal duty to control the conduct of a second person in order to prevent that person from harming a third person (Restatement [Second] of Torts § 315[a] [1965]). Under this rule, psychiatrists traditionally have only owed a duty to third persons for their patients' actions in limited circumstances, such as the negligent discharge of a hospitalized dangerous patient who causes harm to a third person. However, in the main, psychiatrists were not held accountable for the actions of their outpatients, who were alone liable for their own actions. In the wake of the Tarasoff v. Regents of the University of California (1976) decision, the therapist's legal duty and potential liability significantly expanded. In *Tarasoff*, the California Supreme Court reasoned that the special relationship required to recognize a duty to a third party was satisfied in this circumstance based on assumptions the court made regarding the ability of mental health professionals to predict dangerousness. The court held that "the single relationship of a doctor to his [or her] patient is sufficient to support the duty to exercise reasonable care to protect others" from the violent acts of patients.

Other jurisdictions have also concluded that the need to safeguard the public may override confidentiality and call for a psychotherapist to contact the police or the potential victim, hospitalize the patient, or take other measures that would protect a third party. In most states, a psychotherapist owes a legal duty to protect identifiable persons from reasonably foreseeable patient violence. In jurisdictions that recognize such a duty, there are typically requirements that the threat of serious violence be reasonably foreseeable if no protective action is taken and that the victim be identifiable. Several states have rejected recognition of a legal obligation by a doctor to persons injured by the acts of his patient (Evans v. United States 1995; Green v. Ross 1997). For example, in Thapar v. Zezulka (1999), the Texas Supreme Court ruled that the state statute *permits* but does not *require* disclosures by therapists of threats of harm by patients against others (see Quattrocchi and Schopp 2005). Others have enacted legislation to make clear what circumstances trigger the duty and how it may be met. For example, California statutory law now limits a psychotherapist's liability for failure "to warn of and protect from a patient's threatened violent behavior or failing to predict and warn of

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and protect from a patient's violent behavior except where the patient has communicated to the psychotherapist a serious threat of physical violence against a reasonably identifiable victim or victims" (Cal. Civ. Code § 43.92 [2006]). When a duty arises, it may be met by "the psychotherapist making reasonable efforts to communicate the threat to the victim or victims and to a law enforcement agency."

When Does the Duty to Protect Arise?

The duty to protect arises, where it is recognized, to avoid serious violence that would otherwise likely occur without a timely opportunity for other intervention. Thus one dimension of the requirements often imposed for a duty to arise is temporal—the patient must pose an imminent threat of serious harm to an identifiable third party. Whatever legal balance this term strikes between patient privacy and public safety, it is clinically problematic. Imminence of violence is an illusion of short-term prediction for which no risk factors exist (Simon 2006). Determining what the "standards of the profession" are in evaluating an imminent threat (i.e., predicting dangerousness) and how the duty should be carried out is unsettled. As a result, this area of law is controversial and vexing. In jurisdictions that have statutorily specified the circumstances triggering a duty to protect, clinicians have found that a duty to protect that contains more concrete thresholds provides more latitude for treatment interventions. For example, Colorado statutory law provides that liability for a mental health professional who fails to warn or protect is limited to circumstances in which "the patient has communicated to the mental health care provider a serious threat of imminent physical violence against a specific person or persons" (Colo. Rev. Stat. Ann. § 13–21-117 [2006]). Absent such statutory thresholds, courts typically focus on whether the violence was foreseeable and whether a sufficient element of control was present.

Three threshold factors need to be addressed by the clinician in a potential duty-to-protect situation:

1. Assess the threat of violence to another.
2. Identify the potential object of that threat.
3. Implement some affirmative, preventive act.

The law assumes that the foreseeability of violence provides an intended victim an opportunity to avoid harm that the law should value. Thus, if the fact finder determines that but for the psychiatrist's failure to identify or take action to protect against foreseeable violence it would not have occurred,

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the psychiatrist is liable for the harm caused. Foreseeability, however, is a legal term of art that asks what should have reasonably been anticipated. It asks but does not answer questions about the ability of clinicians to predict violent behavior. Foreseeability also should not be confused with preventability. In hindsight, some violent acts seem *preventable* that were not *foreseeable*.

Assessing the Threat

Recognizing the inability to predict the occurrence of violence reliably, the law does not demand that all predictions be correct but instead requires only that psychiatrists exercise reasonable care in making such assessments. This requires compliance with ordinary violence assessment procedures used in the profession. Steps such as taking a thorough pretreatment history, asking about and exploring past or present violence, and attending to factors reasonably thought to contribute to violent behavior are likely to satisfy a reasonable standard of care (see Tables 9–1 and 9–2 later in this chapter).

Identifying the Intended Victim

If the clinician determines that a threat of violence exists, it is clinically and legally necessary to follow up that assessment with a determination of the person(s) who might be the object of the threat. Early cases and an increasing number of recent decisions have held that the duty to protect applies only when there is a “specific or identifiable threat to a specific or identifiable victim” (Brady v. Hopper 1984; Thompson v. County of Alameda 1980; White v. United States 1986). A few courts have broadened this finding by requiring the duty to protect whenever there is a “foreseeable risk of harm” to the public at large (Lipari v. Sears, Roebuck and Co. 1980; Schuster v. Altenberg 1988).

Discharging the Duty

A number of courts and state statutes have addressed what is required to discharge the duty to protect. The statutes usually provide that the duty is met when an identifiable victim is warned and/or the police notified (Appelbaum et al. 1989). Judicial decisions are, by nature, more pragmatic and less comprehensive about how the duty to protect may be met. For example, although the issue was never decided, the therapist in *Tarasoff* had notified the campus police of his concerns. The broad language used in the cases affords a therapist a certain latitude in determining “whatever steps are reasonably necessary” to discharge that duty.

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A conceptual model for assessing violence and implementing suggested psychiatric interventions is outlined in Tables 9–1 and 9–2. The legal reasoning applied in cases involving the duty to warn and protect suggests a number of factors to consider in order to reduce the risk of potential liability, if a patient threatens harm to others.

Special Considerations

Time Limitations

The courts have not clarified important temporal aspects of the duty to protect. Specifically, for how long after the last patient contact will a therapist be held accountable for the patient's act? Rather than drawing lines, the time between the last patient contact and the injury of a third party is simply one factor for the jury's consideration. For example, in Naidu v. Laird (1987), the Delaware Supreme Court upheld a finding that an inpatient psychiatrist was negligent in failing to foresee a former patient's potential to commit a violent act 5½ months after discharge. The court stated that the lapse of time, by itself, was not a bar to recovery but one factor to be weighed by the jury.

Confidentiality and Liability

The recognition of a duty to warn does not eliminate the necessity to resolve an opposing risk of lawsuit for breach of confidentiality in unreasonably discharging the duty to protect (i.e., unnecessary disclosures where a duty to protect exists or any disclosures where a duty to protect does not exist). Because fulfilling the duty to protect through disclosure to the police or hospitalization of the patient helps to prevent patient violence, the question of whether the risk of violence was correctly assessed and the breach of confidentiality justified remains unresolved. At the least, a disclosure must be made with discretion and in a manner that will preserve the patient's privacy while preventing potential harm. Moreover, the disclosure must be based on a well-documented, reasonable risk assessment of violence.

Unforeseeable Violence

Liability for failing to act to prevent third-party injury is not appropriate if the evaluation leading to the conclusion that a patient is at low risk for violence is based on acceptable and reasonable clinical judgment. Although a finding that an evaluation was based on accepted standards for clinical judgment should not result in a finding of liability, some analysis suggests that deviations from clinical practice that may be professionally acceptable are nonetheless troubling to jurors.

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Unreachable Identified Victims

In situations in which the patient is judged to be a threat to a third party, but the third party cannot be reached in order to be warned, the therapist must consider notifying the police or contacting someone in close relationship with the potential victim. If clinically indicated, the patient may require hospitalization. Documenting that the therapist was unable to reach the identified victim and that alternative approaches were used may be important in determining liability.

Evolving Trends

An important, evolving trend is the application of the *Tarasoff* duty to protect third persons from sexual abuse by a pedophile treated by the therapist. In one reported decision, Garamella v. New York Medical College (1998), the appellate court upheld the denial of a psychiatrist's motion to dismiss a tort claim for not reporting to the medical school that his student/patient was a pedophile. The student/patient, a psychiatric resident, engaged in child molestation at a hospital crisis center. Reasoning that the outpatient setting did not present an insurmountable problem for the plaintiff because of the supervising psychiatrist's control over his student, the court left for trial the matter of the plaintiff's foreseeability. A *Tarasoff* duty was also found in a case in which a spouse had knowledge of her husband's sexually abusive behavior against children in the neighborhood (J.S. v. R.T.H. 1998; Touchette v. Ganal 1996). In another case, the court found that a *Tarasoff* duty could exist but declined to find the parents of a babysitter liable for his dangerous sexual behavior (Popple v. Rose 1998). The court determined that no evidence existed that the parents knew of their son's proclivity to commit a sexual assault.

Table 9–1. Systematic violence risk assessment: a conceptual model

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Assessment factors	Risk	Protective
Individual		
Specific person threatened[a]		
Past violent acts[a]		
Accessible victim		
Motive		
Clinical		
Psychiatric diagnosis (Axis I and II)		
Thought insertion or control		
Command hallucinations		
Control of anger		
Syntonic or dystonic violence		
Nonspecific threats		
Childhood abuse (or witnessing parental spouse abuse)		
Alcohol abuse		
Drug abuse		
Mental competency		
History of impulsive behavior		
Neuropsychiatric disorder		
Interpersonal		
Therapeutic alliance (ongoing patient)		
Family, partner, work, other relationships		
Fear of control by others		
Situational		
Stressor(s) specify		
Living environment		
Employment status		
Availability of lethal means (e.g., guns)		

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Epidemiological		
Age		
Gender		
Sociocultural group (base rates)		
Marital status		
Violence base rates		
Overall risk rating:		

Instructions:
1. Rate risk factors present as low (L), moderate (M), high (H), nonfactor (0).
2. Judge overall violence risk as low, moderate, or high.

Note. Tables 9–1 and 9–2 represent a conceptual model of violence risk assessment and intervention. The purpose of these tables is to encourage a systematic approach to risk assessment. The therapist's clinical judgment concerning the patient remains paramount. Because violence risk factors will be assigned different weights according to the clinical presentation of the patient, the method represented in these tables should not be followed rigidly.
[a]When a specific person is threatened and past violence has occurred, the patient is at high risk to commit violence.

Table 9–2. Assessment of violence risk and examples of outpatient psychiatric intervention options

Violence risk	Psychiatric interventions
High	Immediate hospitalization if mentally ill and likely to benefit from hospitalization
Moderate	Hospitalization Frequent outpatient visits Consider warning endangered person(s) and calling the police Reevaluate patient and treatment plan frequently Remain available to the patient
Low	Continue with current treatment plan

Summary

Although the potential for psychiatrists to be held liable for their patients' violent acts toward third parties has expanded, only a relatively small number of psychiatrists and other mental health professionals have been found liable. In 1990, Beck estimated that approximately 50 psychiatrists were sued each year for breach of the duty to protect. Of these cases, roughly two-thirds settled before trial. Of the 17 trials, approximately two-thirds resulted in defendant verdicts. Thus six psychiatrists a year were found liable for violating a duty to protect third parties from their patients. For a psychiatrist who is a member of the American Psychiatric Association (APA; total membership in 1990 of approximately 35,000), Beck estimated that the odds of being found liable for a breach of the duty to protect are 5,800 to 1 in any given year. Nonetheless, prudent practice demands that

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Tarasoff be considered a national standard for psychiatrists and other mental health professionals (Beck 1987).

Clinical Management of Legal Issues

Predicting Violent Behavior

The term *dangerousness* is a legal, not a psychiatric, construct. Accordingly, instead of *dangerousness*, the term *risk of violence* is used in this book. A robust pattern of findings across studies supports a causal connection only between some mental disorders and violence (Monahan 1992). The National Institute of Mental Health Epidemiologic Catchment Area Study estimated that 90% of persons with current mental illnesses are not violent (Swanson et al. 1990). If a person was not having an acute psychotic episode or if psychotic symptoms were not part of the psychiatric problems, the individual was no more likely to be involved in violent behavior than the average person. Even when psychotic symptoms were present, there was only a modest increase in risk of violence above the norm. The persons with the highest risk of violence were young, substance-using males from lower socioeconomic classes. Seriously mentally ill patients with schizophrenia, major depression, mania, or bipolar disorder had an incidence of violence five times higher than that of persons with no diagnosed mental illness. The incidence of violence was 12–16 times higher among persons who were alcohol and substance abusers.

Violent behavior is a function of the dynamic interaction between social, clinical, personality, and environmental factors over situations and time (Widiger and Trull 1994). “Second-generation” research on violence prediction shows that certain elements of the clinical situation contribute to imminent violence by the patient, thereby allowing appropriate interventions to be made. Violence is a function of the dynamic interaction between a specific individual and a specific situation for a given period of time. Nevertheless, because of the lack of clinical standards for the prediction of violence in any context or time frame, the prediction of violence becomes an unreliable exercise.

Stone (1984), however, contended that psychiatrists do have some expertise in determining whether a patient manifests violent tendencies based on the mental status evaluation. Therapists’ predictions of the actual occurrence of violent acts by patients, however, cannot be made with any degree of professional skill. As noted earlier, no short-term risk factors identify the imminence of violent behavior. Nevertheless, Tardiff (1991) maintained that well-trained clinicians should be able to predict a patient’s short-term *potential* for violence by using evaluation techniques similar to those

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used in the short-term assessment of suicide risk. Binder (1999) posed the question, “Which mentally ill, under what circumstances, are dangerous?” Her research data show that although short-term predictions of violence can be relatively accurate, clinicians are better at predicting violence for certain patients and specific acute symptom patterns are related to violent acts.

The notion that therapists have no more skill than laypersons in predicting imminent violence was derived from “first-generation” research studies on the long-term clinical prediction of violent behavior in populations of people convicted of criminal offenses (Binder 1999). It should be noted that a number of these studies, however, were methodologically flawed. Nevertheless, according to these studies, the accuracy level has been roughly one correct prediction out of every three predictions made. These predictions have been limited by emphasis on certain traits of those guilty of criminal offenses, without the benefit of any situational analysis.

Because of psychiatrists’ high false-positive prediction rate, combined with a low base-rate occurrence of violence in outpatient psychiatric populations, accurate prediction of long-term violence to endangered third parties remains elusive. The MacArthur Violence Risk Assessment Study was established to help rectify this situation. Its purpose was to improve clinical risk assessment validity, to enhance effective clinical risk management, and to provide data on mental disorders and violence for informing mental health law and policy (Dix 1980). In this study, violence risk assessments were found to have a validity that was modestly better than chance. Until more studies are available, sound clinical practice requires that thorough violence risk assessments be routinely performed on potentially violent patients, relying on evidence-based violence risk factors.

An accurate assessment of a high risk of violence may be more likely for some patients when the unique interplay of patient and situational factors is psychodynamically understood. Assessment accuracy also can be improved when dealing with defined homogeneous populations with high base rates of violence. For example, a closed psychiatric unit containing very disturbed patients may have a violence base rate of 25%–35%. Epidemiologic data containing known base rates for specific groups are an important assessment variable. Studies that focus on inpatient populations have demonstrated significantly improved predictability (Steadman et al. 1998).

The best protection against allegations of negligence when assessing the risk of violence is to evaluate the patient according to methods and procedures that take into account known violence risk factors (Tables 9–1 and 9–2). Because the clinician is likely to be wrong more often than not, humility and sound clinical judgment dictate careful documentation of how the assessment of violence is conducted.

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In Barefoot v. Estelle (1983), a review of the constitutionality of a state court's imposition of capital punishment, the U.S. Supreme Court reasoned that if jurors are authorized to bear responsibility for a death penalty based on predictions of future behavior, then psychiatrists should not be prohibited from testifying about dangerousness by the Eighth Amendment's ban on cruel and unusual punishment, despite the low predictive ratio of one out of three. Although this may appear to set a judicial benchmark for prediction accuracy, the logic of using this constitutional decision to determine the professional standard of care for risk assessment in duty-to-protect cases addressing a different issue is problematic. Moreover, duty-to-protect issues are creatures of state tort law, in which the U.S. Supreme Court's constitutional authority is rarely pertinent.

Violence Risk Factors

Certain personal, socioeconomic, situational, and clinical risk factors are associated with violence. Monahan (1981, 1991) demonstrated a statistical association between violence and epidemiologic data (see Table 9–3). Tardiff (1988) disputed race as a factor in violence. Studies of patients with and without criminal convictions do not uniformly find a correlation in the incidence of violence with race (Monahan 1981, 1991).

Situational factors associated with violence include an unstable family, a violent environment, a violent peer group, and availability of weapons (e.g., handguns). Some of the risk factors reported to be associated with violence in the psychiatric literature are listed in Table 9–4.

Every study on the assessment of violence risk factors has found that the single factor most highly correlated with the potential for future violence is a history of violence (Tardiff 2002). Violent patients should be asked how they feel about having committed or threatened to commit violent acts. Determining whether violent behavior is syntonic or dystonic for the patient is an essential element of violence risk assessment. The therapeutic alliance can be an effective deterrent to violence. With new patients or patients seen in an emergency, sufficient time may not have elapsed for an alliance to form.

Link and Stueve (1994) found a relationship between a subset of psychotic symptoms and violence, specifically thought insertion, thought control, and the fear of being harmed by others. The index of suspicion for potential violence should be high in patients with acute psychosis who are substance abusing, angry, fearful of being harmed, and experiencing delusions of being controlled or influenced. Substance abuse significantly raises the rate of violence in both patient and comparison groups (Dix 1980). A number of factors have been identified in young persons that should alert

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mental health professionals to an increased potential for future violence (Monahan 1981, 1991; Table 9–5).

Table 9–3. Epidemiologic risk factors associated with violence

- Previous violence
- Sex (males 10 to 1 over females in the United States)
- Race (controversial)
- Younger age
- Employment and residential instability
- History of alcohol abuse
- History of drug abuse

Motive is a very important clinical variable in the assessment of violence (Beck 1987). When the patient responds affirmatively to the question about wanting to hurt someone, the therapist must further ask, “Who?” “When?” “Where?” “Why?” and “For how long?”. Information gleaned from these inquiries is critical to the evaluation of the following: 1) how acutes the threat of violence is, 2) who the object of potential violence is, and 3) what can be done to treat the patient and protect potential victims.

Careful assessment and documentation of pertinent risk and protective factors used in assessing the potentially violent patient is an important way to avoid allegations of negligence. Even if the therapist is wrong in the assessment of the risk of violence, a reasonable mistake is not malpractice. Nevertheless, systematic assessment of the risk of violence is good clinical practice and only secondarily a risk management technique.

Violence risk assessments, both actuarial and clinical, have been compared to weather forecasting (Monahan and Steadman 1996; Simon 1992). The clinical assessment of the risk of violence is a here-and-now (current weather) determination. Clinical assessments of the risk of violence, like weather forecasts, require frequent updating. Actuarial assessments are similar to short- and long-term weather predictions (24–48 hours, 7–10 days). Clinical assessment directs treatment. Actuarial assessment makes a prediction of violence. The assessment model presented in Tables 9–1 and 9–2 is suggested only as a coneptual model and an educational clinical guide. Professional judgment concerning the unique clinical presentation of the patient and the associated

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specific situational factors should dictate appropriate decision making. Accordingly, with the exception of *specific person threatened* and *past violent acts*, violence risk factors in Table 9–2 are not listed in order of clinical importance. That determination depends on the patient's clinical presentation. It is not necessary to reproduce the tables in the patient's chart. A standard note containing an adequate violence risk assessment is all that is necessary. The weather forecast model provides one way of thinking about violence risk assessment. This conceptual model of suicide-violence risk assessment is derived from the author's clinical experience and the evidence-based psychiatric literature. Each practitioner's clinical experience is singular and unique. Competent violence risk assessments will vary accordingly.

Table 9–4. Clinical risk factors associated with violence

- History of violence
- A stated desire to hurt or kill another
- Alcohol and substance abuse
- Inability to control anger (e.g., intermittent explosive disorder)
- Impulsivity (e.g., previous violence toward others or self, reckless driving, unrestrained spending, sexual promiscuity)
- Paranoid ideation, thought insertion or control, fear of harm
- Command hallucinations
- Psychosis
- Personality disorders: antisocial, borderline
- "Soft" neurological signs (e.g., neuropsychiatric disorders)
- Substance abuse
- Mental retardation

Other approaches to violence risk assessment exist (Monahan 1991). Actuarial analysis is useful in identifying diagnostic groups at higher risk for violence rather than for trying to predict the violent acts of a specific patient (Monahan et al. 2001). Actuarial analysis does not identify patient-specific treatable risk and modifiable protective factors. Actuarial instruments are not sensitive to patients' clinical changes that guide treatment interventions or gauge the impact of treatment (Norko and

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Baranoski 2005). No method of violence risk assessment can predict who will commit violent acts and when. Regardless of which method is used, the important point is that in the management of violent patients, violence risk assessments need to be made and recorded in the medical chart as required.

Table 9–5. Risk factors in young persons associated with increased potential for violence

- Violence as a juvenile
- Conduct disorder
- Psychosis
- Substance abuse or dependence
- Neurological impairment or head injury
- Childhood abuse
- Witnessing parental spouse abuse
- Psychiatric hospitalization
- Mental retardation

Demonstration of Violence Risk Assessment

After doing a complete psychiatric examination, the clinician should perform, record, and document a competent violence risk assessment. The patient's risk factors should be rated as low (L), moderate (M), high (H), or nonfactor (0). This dimensional model method assesses risk factor severity. Risk factor severity is a measure of intensity and duration (acute or chronic). For example, does the patient experience fleeting thoughts of violence or escalating impulses for violence? The rating of protective factors against violence include internal and external resources, for example, adequate coping skills and family support. After weighing risk factors, the overall assessment of violence risk should be rated as low, moderate, or high. The following case example illustrates the importance of assessing a patient's violence risk at the time of a contemplated discharge from a psychiatric unit. After reviewing the case example, the reader is encouraged to do an independent violence risk assessment before reading the psychiatrist's assessment.

> A 35-year-old patient with chronic schizophrenia is admitted to the psychiatric unit of a general hospital because of auditory hallucinations commanding him to kill his mother. He also expresses the delusion that his mother is poisoning

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him. The patient has not lived apart from his mother. He works sporadically as a laborer. His only income is from Social Security Disability Insurance. The patient is treated with antipsychotic medication and individual and group therapies. Because the patient speaks only broken English, verbal interaction between the psychiatrist and the patient is minimal. The psychiatrist relies on observation of the patient's behavior on the ward and the input from the treatment team.

The psychiatrist learns from the patient's mother that the patient has been involuntarily hospitalized three times in the past 10 years. An assault preceded each of the previous hospitalizations. Before the second hospitalization, the patient stabbed a cousin. He was criminally charged with felonious assault but found not guilty by reason of insanity. The psychiatrist feels that she needs more information. She asks the unit secretary to call and request fax copies of the patient's previous hospital summaries.

After 10 days of hospitalization, the psychiatrist reviews the clinical course of the patient with the staff. The patient stays to himself most of the time. There is no evidence of any violent behavior. He is described in the nursing notes as "very cooperative." He complies with ward routine and regulations. Medication is taken. After the fourth day of hospitalization, the patient denies hallucinations or delusions. The staff informs the psychiatrist that the patient becomes sullen and withdrawn when his mother visits. No off-ward privileges are requested by the patient.

A discharge conference is held with the treatment team. The conference participants conclude that the patient is ready for discharge. The patient denies any intent to harm his mother or anyone else. At the discharge conference, the just-received fax summaries of the previous hospitalizations are reviewed. The summaries reveal that the patient was diagnosed with paranoid schizophrenia on each admission. The history is also consistent for each hospitalization. The patient fails to keep his outpatient visits, stops taking his medication, and begins drinking. He then becomes delusional, begins to hear voices, and becomes assaultive. The psychiatrist and the treatment team are concerned about the patient's revolving-door history of paranoid schizophrenia, alcohol dependence, noncompliance with treatment, decompensation, violence, and rehospitalization. The patient's history is similar to that of a number of other patients with schizophrenia admitted to the unit. The treatment team concludes that the patient has received maximal therapeutic benefit from inpatient treatment. The patient has one more hospital day authorized by the managed care organization. The psychiatrist performs a violence risk assessment, judging that the patient's overall lifetime risk of violence is high but is low *at the time of discharge* (see Table 9–6). The violence risk assessment is included in a risk-benefit assessment that weighs the risks and benefits for both continued hospitalization and discharge. The risk-benefit assessment favors discharge.

The psychiatrist is careful to note that the violence assessment is a here-and-now evaluation. At the time of discharge, the patient's risk of violence is assessed as low. The patient, however, is at a high risk for future violence if there is an exacerbation of his severe psychiatric disorder, noncompliance with treatment, alcohol abuse, an altercation with his mother, and other unforeseen, situational stress factors.

Psychiatric facilities for long-term hospitalization are not available. Because the patient has progressed so well, the psychiatrist decides to discharge the patient. The psychiatrist discusses the discharge with the patient's mother. The psychiatrist advises the mother to remove any potentially lethal weapons from the home and requests a callback to confirm their secure storage elsewhere. She agrees but expresses trepidation to have her son live at home. She hopes her son will be able to go back to his job as a laborer. The patient rejects referrals to Alcoholics Anonymous and the hospital's day treatment program. The social worker arranges an appointment for the patient at the community outpatient center for the first available session. His appointment is scheduled for 1 week after discharge. The patient is seen on the day of discharge and is stable and eager to leave.

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This case example has been presented at clinical conferences where clinicians were asked to assess the patient's risk of violence and the appropriateness for discharge. The clinicians were clustered into two groups on the violence risk rating. All agreed the patient was at high risk of future violence. Disagreement arose, however, about the psychiatrist's low violence risk rating at the time of discharge. Some clinicians (Group A) felt that the distinction between the low risk of violence at the time of discharge versus the patient's high risk of future violence was a distinction without a difference. This group explained that the patient was at just as high a risk of violence at discharge. They observed that, as expected, the patient functioned quite well in a structured hospital environment where he was monitored for medication compliance. It was entirely foreseeable, however, that the patient would repeat the prior cycles that led to violence and rehospitalization. These clinicians recommended that the patient be involuntarily hospitalized and then transitioned to outpatient commitment status to ensure follow-up and compliance.

Table 9–6. Demonstration of a rapid, competent violence risk assessment.

Assessment factor	Risk	Protective
Specific person threatened	0 (in hospital)	0
Past violence	H (stabbed cousin)	
Accessible victim	H (mother)	
Therapeutic alliance		L–M (with staff)
Psychiatric diagnosis	H	
Command hallucinations	0 (in hospital)	0
Employment		L
Specific plan	0	0
Treatment response		H
Medication compliance		H (in hospital)
Structured environment		H
Alcohol dependence	H	
Substance abuse	0	0
History of impulsivity	H	
Guns		L (none at home)
Compliance with aftercare	H	
Relationships	M–H (loner)	
Overall risk rating:	Low (at discharge)	

The other group of clinicians (Group B) felt that Group A's recommendations were impractical. The purpose of hospitalizing a patient in the managed care era was rapid stabilization and referral to outpatient treatment. Inpatient psychiatrists provide acute care for very sick patients, similar to the

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intensive care unit for medical-surgical patients. Involuntary hospitalization would be inappropriate because the patient would not meet the standards for involuntary commitment, especially the usual requirement of “imminent” danger to self and others. Moreover, hospitals receiving involuntary patients discharge them rapidly if the patients are stable.

Long-term private psychiatric hospitalization was not an option for this patient who, except for receiving Social Security disability benefits, was impecunious. Group B felt that the best course of action was to carefully structure outpatient follow-up for compliance with treatment. Because the potential for violence toward the mother existed, referral of the patient should be made to a supervised residential setting where his condition and medication could be monitored. The conferees were in agreement that managed care pressures should not lead to a premature discharge. A lively debate invariably occurred about clinicians’ responsibility for continuing care of an unstable inpatient after a managed care organization refuses to authorize payment for additional stay. Younger clinicians with families to support had greater difficulty in assuming the financial loss of caring for acutely ill patients until they were stabilized and discharged.

The *Tarasoff* Duty and Psychiatric Practice

The *Tarasoff* duty of the therapist to protect foreseeable victims of patient violence applies to outpatient as well as inpatient cases. In treating an outpatient, the *Tarasoff* duty generally is triggered by a patient’s expressed or implied threats or acts directed to a specific, foreseeable victim. The violence threatened must be substantial, involving serious bodily harm or death. If no threats or violent acts are uncovered after careful clinical evaluation, liability is unlikely even if violence occurs.

In inpatient release cases, the duty to protect does not require a specific foreseeable victim as long as there is reasonable evidence that the patient may be dangerous. An appropriate evaluation of the patient for potential violence satisfies the *Tarasoff* duty. If hospitalization is based on a court order and the court releases a patient deemed to pose a high risk of violence, the psychiatrist should attempt to make known his or her concerns about the patient’s potential for violence. However, the judicial decision to release insulates the psychiatrist from liability.

In some states with statutory provisions limiting the *Tarasoff* duty to warning an endangered, identifiable victim, the clinician may be distracted from providing the full spectrum of clinical

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interventions (Klassen and O'Connor 1994). Warning, by itself, is often insufficient as a clinical intervention. Usually, more needs to be done clinically. Whenever possible, the *Tarasoff* duty should be integrated into the clinical work with the patient.

Beck (1985) noted that once the clinical assessment is made that a patient is potentially violent, three basic options are open to the clinician: 1) deal with the violence in the therapy, 2) warn the victim and/or the police, or 3) hospitalize the patient voluntarily or involuntarily.

Systematic assessment of risk and protective factors informs the therapist's clinical options. Accordingly, potentially violent patients should be seen face-to-face frequently and their risks for violence reassessed at each visit. Documentation that a violence risk assessment was conducted evidences adherence to the standard of care. Violence risk assessment is, however, a process, not an event. Clinicians cannot provide a guarantee of accuracy, only an assurance of the process of assessment.

Clinical Considerations

Long before *Tarasoff*, therapists warned endangered third persons as part of their professional and ethical duty when they possessed "insider information" from treatment about the risk of violence posed by a patient to others. The duty to warn should not blind the clinician from implementing other clinical interventions that may be more effective in meeting their professional and ethical duties. The great majority of potentially violent patients can be managed through good clinical practice.

Warning Endangered Third Parties

Simply issuing a warning as a legal formalism is not an acceptable approach to managing the violent patient. Although a number of states have defined the *Tarasoff* duty in statutes and have narrowed the potential liability, the therapist who does not exercise reasonable professional judgment in managing the violent patient may still be vulnerable to a lawsuit by the patient for negligence. Clinical efforts to manage and control the patient should be attempted before a warning is issued to the intended victim. Frequently, the patient's treatment ends when a warning is made to an endangered third party. At this point, the therapist's intervention options are severely limited or nonexistent. Involuntary hospitalization may be an appropriate, last-resort intervention for selected patients who meet the substantive criteria (mentally ill and dangerous) for civil commitment.

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Therapists have a moral, ethical, and professional duty to protect patients and their potential victims. The imposition of a legal duty is incidental to the therapist's professional duty. When the duty to protect is implemented, every effort should be made to include the patient in the process.

Giving a warning to meet the duty is not without peril. Many warnings will be based on erroneous predictions, unnecessarily alarming the person warned as well threatening the therapeutic alliance between patient and therapist. Warnings based on accurate assessments provide neither assurance of police protection nor comprehensive guidance to avoid the risks posed by the patient.

In general, if the therapist decides to warn, a phone call is appropriate. A phone call allows the potential victim to ask questions. Nuances and difficulties in communication can also be appreciated by both parties. Telephoning in the patient's presence may help to temper exaggerated remarks by a therapist's overreaction to the threat of violence. In addition, the patient's presence tends to head off suspicions or outright paranoid ideas that the psychiatrist is acting duplicitously. Sometimes, a trusted third party may act as a go-between. The warning should be made clearly. The clarity of the warning has been open to second-guessing by some courts. Successful malpractice suits are unlikely when good-faith warning and reporting are based on reasonable clinical judgment.

The psychology of victimization is pertinent to the decision to warn (Monahan and Steadman 1994). For example, the psychodynamics of victim–victimizer, husband–wife, or partner relationships in which one party is being abused may dictate attempting to move the victim toward a sheltered environment rather than warning the abuser that the abused partner is considering retaliatory violence. To do otherwise might seriously endanger the patient or precipitate mutual violence.

How the warning is given, not *whether* one is given, is sometimes the more critical factor. When the clinician discusses the warning with the patient before giving it, generally the clinical result is positive. Failing to discuss the warning with the patient often harms the therapeutic alliance and the therapy. Potential victims should be warned in a clinically supportive manner. If they feel that evasive action can be taken and that the therapist is acting in a responsible, genuinely concerned manner, the warnings are likely to be received positively.

Generally, courts have been reluctant to impose a duty on psychiatrists to warn the foreseeable victim when the latter knew of the potential for violence from a patient (Lewis et al. 1988). For example, in Estate of Heltsley v. Votteler (1982), the plaintiff had knowledge of the patient's previous aggressive behavior but contended that a warning from the psychiatrist would have made her appreciate the significance of the threat. The Iowa Supreme Court upheld the trial court's

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summary dismissal, reasoning that the duty should not be imposed when the foreseeable victim knows of the danger. Not all cases place the same emphasis on the victim's knowledge. For example, in Jablonski v. United States (1984), a child brought a wrongful death claim against the government under the Federal Tort Claims Act when her mother was killed by her boyfriend, a patient who was being evaluated at a Veterans Administration hospital, after making earlier threats to the mother. In a judge trial the court found, and the appellate court agreed, that the plaintiff had met the requirements to impose liability under *Tarasoff*: 1) the existence of a psycho-therapist–patient relationship; 2) that the psychotherapist knew, or should have known, that the patient was dangerous; 3) that the mother was a foreseeable victim of the patient's violence; and 4) that the psychotherapist did not take the necessary steps to discharge his duty (i.e., failed to secure the patient's prior medical history and to warn the mother that the patient's violence may be directed toward her). Although the victim had been the recipient of the threats for which the patient was being evaluated, the matter of what duty is owed to a knowledgeable victim was not raised by the parties or the court.

The therapist should not assume that an endangered person who has been previously threatened or harmed by the patient fully appreciates the current danger of his or her situation. Denial may cause the person to minimize or ignore the threat. It may be necessary to warn the endangered person of the specific threats made by the patient, despite his or her prior knowledge.

Confidentiality Concerns

Trust is the cornerstone of psychiatric treatment. Without trust, no therapeutic alliance can develop, damaging the patient's chances of receiving psychotherapeutic help. In the real world, however, confidentiality, like trust, cannot be absolute. Exceptions to the maintenance of confidentiality exist for the protection of both the patient and society.

In *Tarasoff*, the court was mindful of the importance of maintaining as much confidentiality as possible in the therapist–patient relationship. The court stated that warning of a victim should be done in such a way as to preserve confidentiality consonant with the prevention of threatened danger. Confidentiality should not be breached by warning a third party unless the threat is serious and substantial, an identifiable victim is endangered, and there is no alternative way to fulfill the duty to protect (e.g., hospitalization). Nevertheless, a plaintiff may allege breach of confidentiality.

The Principles of Medical Ethics With Annotations Especially Applicable to Psychiatry (American Psychiatric Association 2001) states, "Psychiatrists at times may find it necessary, in order to protect the patient or the community from imminent danger, to reveal confidential

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information disclosed by the patient" (p. 8, Section 4, Annotation 8). The *Tarasoff* exception to confidentiality is part of the same public policy exception requiring the reporting of contagious diseases, suspected child abuse, and gunshot wounds.

Potential conflicts between the duty to maintain confidentiality and the duty to protect may also arise concerning the capacity of a patient to operate an automobile (Pettis 1992). A duty to protect third parties might demand notification of the bureau of motor vehicles regarding psychiatric patients who should not drive. Indeed, by statute, a number of states require physicians to report cognitive or functional impairments likely to affect the patient's ability to operate an automobile safely (Or. Rev. Stat. § 807.710 [2006]). Another related issue is the psychiatrist's accountability for injuries to third parties when a patient was not informed of medication risks while driving (Levin and Hill 1992).

There is a split of authority regarding the liability of a physician to non-patient third parties injured as a result of physician negligence in the prescription of medication to a patient (Simon 1992). Although some states have concluded that the driving behavior of outpatients is too remote for a physician to control (Estate of Heltsley v. Votteler 1982), other states have found the injuries to a third person a foreseeable risk of failing to inform a patient of the risks of medications that can cause drowsiness (Kaiser v. Suburban Transp. Sys. 1965).

Although the *Tarasoff* duty arises only occasionally, the maintenance of confidentiality is a constant duty in clinical practice. In 1996, the U.S. Supreme Court ruled in Jaffee v. Redmond (1996) that confidential communications between psychotherapist and patient are privileged from compelled disclosure in federal cases under the Federal Rules of Evidence. Perlin (1999) pointed out that a footnote in *Jaffee* states an exception to the privilege where a serious threat of harm exists to the patient or others that "can be averted only by means of a disclosure by the therapist" (518 U.S. at 18 n.19). Perlin commented that "the relationship of this footnote to the *Tarasoff* doctrine has not yet been fully explored" (pp. 20–21).

To further informed consent, some therapists give *Miranda*-type warnings to new patients about the therapist's duty to protect third parties against harm from violent patients. Others conclude that starting treatment in this way may cast a pall over the fledgling therapeutic process. Patients already frightened of their own aggression may find such a warning confirmatory of their worst fears. Secretive patients may seize upon the warning and withhold verbal expression of violent feelings. When the protection of others from patient violence is necessary, clinical interventions may allow for the preservation of confidentiality.

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If a patient gives the therapist good reason to believe that a warning should be issued to an endangered third party, the confidentiality of the communication that gave rise to the warning may be lost. In some states, the warning of endangered third parties has resulted in psychiatrists being compelled to testify in criminal cases, although the matter is unresolved in most states (Leong et al. 1992).

Protecting Endangered Third Parties

Whereas *Tarasoff I* would have recognized a duty to warn, *Tarasoff II*, which vacated the earlier opinion, demanded that the therapist exercise his or her own best judgment consistent with that reasonable degree of skill, knowledge, and care ordinarily exercised by therapists under similar circumstances to *protect* the victim from the foreseeable violence of dangerous patients. The court did not require unerring accuracy but only the skill exercised by therapists in similar circumstances. The duty to protect permits a broader clinical approach to the management of violent patients. Although warning may be part of a therapist's intervention strategy, it rarely should be relied on initially or exclusively. Some clinical options include increasing patient appointments, adjusting medications, consultation, and voluntary or involuntary hospitalization.

The Patient With HIV

With the universal concern about the spread of AIDS, psychiatrists who treat patients testing positive for HIV face special ethical and legal dilemmas. For example, how should the psychiatrist manage the patient who is HIV positive and continues to have multiple sexual relationships but refuses to inform his or her partners? Consent between adults is a sham under this circumstance. Unlike other potentially dangerous patients, the patient who is promiscuous and has concealed HIV infection constantly carries a potentially lethal weapon. The occasion for harm commonly occurs within the unguarded embrace of a sexual relationship. Eth (1988) noted four clinical situations in which violating confidentiality to protect life is ethically indicated: 1) communicable disease, 2) child abuse, 3) threat of violence, and 4) HIV seropositivity.

Psychiatrists treating patients with diagnosed HIV infections who pose a continuing danger of infection to others should evaluate these patients in the same manner as other potentially dangerous patients. It is the patient's behavior, and not the HIV status per se, that represents the immediate danger. The psychiatrist should focus the evaluation particularly on the following questions:

- What is the patient's psychiatric diagnosis (Axis I and Axis II)?

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- Is the patient treatable by available psychiatric therapies?
- What is the motivation of the patient for not informing sexual partners of HIV-positive status?
- Can the endangering behavior be contained by and within the treatment?
- Can support groups and organizations be mobilized to help the unemployed, isolated HIV patient?

Often, patients who are HIV positive are unemployable, shunned by society, alone, and terrified. Sexual contact with others may be motivated not so much by sexual interest as by the need for human contact, affection, and support. To expect these patients to be sexually abstinent without providing other sources of emotional sustenance is naive and insensitive. Support groups and organizations are critically important.

In some cases, the patient may be motivated to infect others out of malice or a callous indifference to the welfare of others. If this behavior cannot be managed clinically, serious consideration must be given to telling the patient that a warning will be issued, if the identity of the endangered parties is known. A report to the health department should also be considered if the authorities will conduct tracing and notification. A full notation of all measures taken should be recorded in the patient's chart. Consultation with a colleague should also be considered. As in other *Tarasoff*-type situations, warnings must be discreet, balancing the right of society to be protected from disease against the psychiatrist's duty to maintain patient confidentiality.

Successful malpractice suits for breach of confidentiality are possible, even when good-faith warning and reporting are based on reasonable clinical judgment. Clinicians should become familiar with health regulations and laws governing the reporting of HIV status. Immunity from liability for health care providers may be available, similar to that found in mandatory reporting requirements currently applicable for other contagious diseases.

The APA's Commission on AIDS has revised its confidentiality and disclosure guidelines to permit psychiatrists to notify an endangered identifiable third party at risk from a patient who is unable or unwilling to take precautions, including abstinence (Lester v. Hall 1998). Other important issues covered in the APA guidelines include 1) the use of involuntary hospitalization when an HIV-positive patient's endangering behaviors are the result of mental illness that can be treated by hospitalization, and 2) the permissibility of notifying public health authorities of previously exposed individuals who are no longer in contact with and at risk of exposure to the HIV-positive patient, if the patient is unable or unwilling to cooperate. As with all warnings given to endangered third

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parties, the psychiatrist must consider the serious emotional impact and adverse consequences that notification may have on the person warned. Appelbaum and Appelbaum (1990) provide an in-depth analysis of confidentiality versus the duty to protect in the management of the patient with HIV.

Assessing and Managing the Risk of Violence

Assessing and managing the risk of violence involves five basic steps (see Tables 9–1 and 9–2):

1. Identify patients with risk factors associated with violence.
2. Assess the overall risk of violence based on the rating of risk and protective factors.
3. Implement treatment and preventive interventions that are informed by the overall violence risk assessment.
4. Evaluate the effectiveness of treatment and other interventions.
5. Continue the process of violence risk assessment.

Based on the assessment of risk and protective factors, a probability determination of low, moderate, or high potential for violence can be made according to a tabulation of clinical factors that increase or decrease the risk of violence. The assessment of the patient's overall risk of violence is based on clinical judgment. No clinical standards exist for predicting the occurrence of a violent act.

Psychiatrists are not held liable for mistakes in assessing the risk of violence per se. Rather, liability is imposed for failing to properly collect necessary data and to adequately assess the risk of violence. The method illustrated in Tables 9–1 and 9–2 may be used when time and circumstances do not permit obtaining a consultation (Perlin 1999).

The nexus between mental disorders and potential violence allows for clinical interventions. Psychiatrists possess the ability to treat and manage most patients who are currently or potentially violent. Potential violence rather than dangerousness is a preferable clinical focus for therapists. Legal definitions of *dangerousness* tend to be arbitrary and abstract concepts not readily translatable into the diagnostic and treatment models used by psychiatrists.

The assessment of potential violence involves two major steps: gathering information and assessing the risk of violence. Appelbaum (1985) suggested that clinicians routinely ask patients two questions: "Have you ever seriously injured another person?" and "Do you ever think about harming someone else?" These two questions often yield surprising information that is not usually obtained from a general psychiatric history unless asked. Areas of assessment include the individual, social,

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situational, and clinical variables related to violence discussed earlier. The most common mistake made by clinicians is to base a violence risk assessment on insufficient information. Although a standard of care does not exist for predicting violent acts, clear standards do exist for gathering sufficient information to satisfy a reasonable standard of care in the assessment of the *risk* of violence. Because the prediction of violence is not possible, careful documentation from visit to visit of the clinical reasoning behind the overall violence risk assessment is essential. Violence assessment is a process, not an event.

The Risk-Benefit Assessment

With any treatment intervention undertaken for health care of a potentially violent patient, a risk-benefit assessment should be conducted and recorded in the patient's chart immediately. In the event of a violent outcome, a documented risk-benefit analysis that considers clinically relevant interventions with potentially violent patients will be useful in demonstrating that reasonable care was taken with the patient. Absolute statements that a patient is no longer at risk of violence are not clinically defensible. Considering risks alone leads to unduly defensive practices. Risk-benefit assessments bring a balanced perspective to clinical decision making.

Risk-benefit assessments should evaluate the risks and benefits both for and against the intervention under consideration. For example, in considering a hospital discharge for a patient who has displayed a potential for violence, the risks and the benefits of the discharge must be considered against the risks and the benefits of continued hospitalization. A note should include a statement about the likelihood of violence, a proposed course of action, and the clinical decision-making rationale.

Consultation with a colleague may be reassuring to both the patient and the therapist. The consultant should document his or her findings. If time does not allow for the patient to be seen, an informal consultation with a colleague can take place over the phone. Although not ideal, such a consultation is better than none at all. The psychiatrist should summarize the consultant's opinion in the patient's record. If legal questions arise later, consultation may help establish that a reasonable standard of care was provided to the patient.

Psychiatric residents should consider increasing the frequency of their clinical supervision when a dangerous patient is undergoing a crisis. Decisions regarding management of the potentially violent patient should be endorsed by the supervisor's signature.

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Treatment Refusal

If a potentially violent patient stops taking medication or drops out of treatment, the patient's continued need for treatment should be addressed aggressively. Voluntary or involuntary hospitalization may need to be pursued, depending on the clinical condition of the patient and the assessed risk of violence. The involvement of family members may also be necessary. If the potential for violence is high, endangered third parties and law enforcement agencies may need to be informed. A telephone call or letter (if time permits and if the patient can be located) may be necessary to determine the patient's treatment intentions. If the patient terminates treatment but is not an acute threat to others, a letter should be sent confirming that the patient has terminated treatment unilaterally. The letter may also recommend continued treatment and state the psychiatrist's willingness to provide referral sources and the patient's records to other treaters, on proper authorization (see Chapter 2, "The Doctor–Patient Relationship"). If the patient is transferred to another facility, the assessment of potential violence needs to be communicated to the new treaters. Reassessment of the risk of violence should be made before discharge (see Table 9–6).

When involuntary hospitalization is sought, the final dispositional responsibility rests with the judicial system. If the psychiatrist recommends commitment and the court finds the patient not committable, and the patient subsequently causes harm, the court is not liable for the consequences of the decision not to commit. Similarly, if an appellate court overturns a commitment, neither the committing court nor the psychiatrist is liable for the commitment. Involuntary hospitalization should be pursued when it is in the best interest of the patient (e.g., if the patient is likely to receive benefit from hospitalization) or for the immediate safety of the patient or others. When a less-restrictive alternative is unavailable, clinicians should not shrink from seeking involuntary hospitalization as an important clinical intervention for patients who meet commitment criteria (see Chapter 7, "Involuntary Hospitalization"). However, involuntary hospitalization should not be implemented defensively by the therapist in an attempt to avoid liability. Seeking involuntary hospitalization solely as a risk management technique will likely destroy the treatment relationship. It will also adversely affect future treatment endeavors with the patient, paradoxically increasing the risk of the therapist being sued.

Release of Hospitalized Patients

Voluntary patients who are at risk for violence may not meet the criteria for involuntary hospitalization. If the patient will be discharged, a careful note explaining the decision-making

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process is essential. Individuals who were threatened or harmed by the patient before hospitalization should be given advance notice of the patient's impending discharge, if the patient is still considered to be at risk of violence. The clinician should inquire about the presence of guns or other lethal weapons at home or easily accessible elsewhere. A responsible, designated person should be advised to remove all lethal weapons and to safely secure them ouside of the home. A callback is necessary to confirm that the instructions were followed.

Appointments need to be scheduled for as soon after discharge as possible. Violent patients are often noncompliant with treatment recommendations. A Veterans Administration study of outpatient referrals found that of 47 inpatients referred to a Veterans Administration mental health clinic, 21 did not keep their first appointments (Zeldow and Taub 1981).

Every discharge is a complex process that must be tailored to the patient's individual treatment needs and circumstances (Felthous 2004). A risk-benefit assessment should be recorded that evaluates the risks and benefits of continued hospitalization versus discharge. Factors to be considered and weighed in the risk-benefit assessment include the risk of violence (against self or others), the severity of illness, the likely compliance with follow-up care, the availability of family or other support, the presence of substance abuse and/or other co-morbid conditions, the need for safety, the importance of resuming life outside the hospital, and other significant individual factors. The absence of violent thoughts, feelings, or impulses is not as important at the time of discharge as an assessment of the patient's ability to control such impulses and the sufficiency of the environment to support the patient's self-control. The majority of patients manifesting violent ideation are treated as outpatients. Clinicians can avoid discharges that are doomed to fail by asking themselves and the clinical staff what is different about the patient's condition and life situation at discharge as compared with those aspects at admission.

Discharge decisions must be carefully documented. Notes should be recorded contemporaneously with decision making. After-the-fact notes are of little value and are legally precarious. A well-reasoned, clearly documented risk-benefit note that reveals the psychiatrist's clinical thinking at the time of discharge will help preempt second-guessing by a court if a lawsuit is later filed. Assessing the risk of violence toward self or others is a here-and-now determination performed at the time of discharge. Once the patient is discharged, the potential for violence will depend on the patient's mental state as well as on concurrent situational factors at any given time. As previously noted, violent behaviors are the result of dynamic, complex interactions among a variety of clinical, personality, social, and environmental factors whose relative importance varies across time and situations.

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The clinician's obligation is to structure the follow-up so as to encourage adherence to the outpatient treatment plan. There are limits to psychiatrists' powers to ensure adequate patient follow-up. These limitations must be acknowledged by the psychiatric and the legal communities. Most discharged patients retain the right to refuse further treatment. The American Medical Association Council on Scientific Affairs has developed evidence-based discharge criteria for safe discharge from the hospital (American Medical Association 1996).

Managed Care and Premature Release

The treatment of psychiatric inpatients has changed dramatically in the managed care era. Most psychiatric units, particularly in general hospitals, have become short-stay, acute-care psychiatric facilities. Generally, only suicidal, homicidal, or gravely disabled patients with major psychiatric disorders pass strict precertification review for hospitalization. Approximately half of these patients have comorbid substance-related disorders. Usually, in order to gain precertification for admission in managed care settings, patients must meet "medically necessary" criteria whose stringency often equals or exceeds that of the substantive criteria for involuntary civil commitment.

The purpose of hospitalization is to provide crisis intervention and management to stabilize severely ill patients and to support their safety. Close scrutiny by utilization reviewers, however, permits only brief hospitalization for these patients. In addition, hospital administration may exert pressure on the psychiatrist for early discharge to maintain length-of-stay statistics within predetermined limits. Increasingly, the treatment of these patients is being provided by a variety of mental health professionals. Yet the psychiatrist must often bear the ultimate burden of liability for treatments gone awry. Limited opportunity exists during the hospital stay for psychiatrists to develop a therapeutic alliance with their patients. The ability to communicate with patients, the psychiatrist's stock-in-trade, is often severely curtailed. All of these factors contribute to a greatly increased risk of malpractice lawsuits against psychiatrists that allege premature or negligent discharge of patients due to cost-containment policies (Simon 1997).

Psychiatrists have certain responsibilities and legal duties to patients treated in managed care settings. These include disclosure of all treatment options, exercise of appeal rights, continuance of emergency treatment, and reasonable cooperation with utilization reviewers. Fulfillment of these duties is important in the management of the potentially violent inpatient. The duty to protect endangered third parties will likely arise with increased frequency as cost-containment measures tighten to curtail the length of patient hospitalization.

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Psychiatrists are being held to a new standard of efficiency by third-party payers. They are expected to rapidly gather patient information, to determine a diagnosis, and to develop a multidisciplinary treatment plan. Psychiatrists must acquire new treatment skills for the rapid management and discharge of potentially violent patients. In addition, they need to have the personal and professional skills necessary to collaborate effectively with the multidisciplinary team in managing violent patients. Psychiatrists practicing in the managed care era are expected to be knowledgeable in the uses and limitations of a number of treatment modalities and behavioral techniques and in the use of medications to achieve rapid improvement and stabilization. In addition, psychiatrists are expected to provide competent, efficient clinical care despite stringent managed care restrictions on insurance coverage for psychiatric treatment. Psychiatrists have the responsibility for, but not necessarily the control of, treatments provided to patients.

Managed care organizations generally limit or deny payment for services but not the actual services themselves. How a patient is treated is strictly the physician's decision. Although the *quality of care* may be adversely affected by managed care restrictions, the *standard of care* for treating self-destructive or potentially violent psychiatric inpatients has not changed. As a result, psychiatrists often feel that they are practicing under conflicting standards (Simon 1998). As in the past, psychiatrists will be judged by whether they fulfill their professional, ethical, and clinical duties to the patient.

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